Pulmonology Service Line Handbook

MERIDIAN HEALTH PARTNERS ## OPERATIONAL POLICY MANUAL: PULMONOLOGY SERVICE LINE Document ID: MHP-POL-PULM-402 Effective Date: January 15, 2024 Applies To: All Pulmonology Department Staff, Central Scheduling, and Referral Coordinators across all 14 Meridian Health Partners sites. Policy Lead: Dr. Aris Thorne, Operations Policy Lead, Meridian Health Partners

SECTION 1: DEPARTMENT SCOPE AND CONDITIONS ACCEPTED

1.1 The Pulmonology Department at Meridian Health Partners provides outpatient consultation, diagnostic testing, and chronic management for respiratory and sleep-related conditions across all 14 clinic locations.

1.2 Accepted Conditions: The department accepts referrals and scheduled visits for the following confirmed or suspected primary diagnoses:
* Asthma (ICD-10 J45 series)
* Chronic Obstructive Pulmonary Disease, including chronic bronchitis and emphysema (ICD-10 J43-J44 series)
* Obstructive and Central Sleep Apnea (ICD-10 G47.3 series)
* Bronchiectasis (ICD-10 J47)
* Interstitial Lung Disease and Pulmonary Fibrosis (ICD-10 J84 series)
* Pulmonary Hypertension, WHO Groups 1 through 5 (ICD-10 I27.0, I27.2 series)
* Chronic Cough exceeding eight weeks duration of undetermined etiology

1.3 Explicitly Not Accepted Conditions: The Pulmonology Department does not accept direct outpatient referrals for, and will immediately redirect or refuse, the following patient profiles:
* Acute respiratory failure requiring mechanical ventilation or immediate inpatient admission.
* Active, untreated pulmonary tuberculosis (ICD-10 A15-A16 series); such patients must be referred directly to the county public health tuberculosis clinic.
* Pediatric patients under the age of 12 years; pediatric pulmonary care is outside the clinical scope of adult multi-specialty outpatient operations at Meridian Health Partners.
* Massive hemoptysis defined as expectoration of greater than 200 mL of blood within a 24-hour period; these cases require emergency department triage.

SECTION 2: APPOINTMENT DURATION AND SCHEDULING TIMELINES

2.1 Standardized appointment durations are mandatory across all 14 Meridian Health Partners sites to maintain master schedule integrity. Schedulers shall allocate time as follows:
* New Patient Consultation: Exactly 45 minutes per encounter.
* Established Patient Follow-Up: Exactly 20 minutes per encounter.
* Diagnostic Pulmonary Function Testing (PFT) Only: Exactly 30 minutes per encounter.

2.2 Named Exceptions: Patients scheduled under the Interstitial Lung Disease (ILD) pathway managed by designated pulmonologist Dr. Helen Vance receive an allocated initial consultation window of 60 minutes to account for multi-system chart review and caregiver integration.

SECTION 3: PATIENT PREPARATION INSTRUCTIONS

3.1 Central Scheduling and the referring intake team must issue the following standardized preparation instructions to patients prior to diagnostic pulmonary function testing:

* Bronchodilator Withholding: Short-acting beta-agonists (e.g., Albuterol) must be withheld for a minimum of 6 hours prior to testing. Long-acting bronchodilators (e.g., Tiotropium, Salmeterol) must be withheld for 24 hours prior to testing.
* Substance Restrictions: Patients must refrain from smoking tobacco or consuming caffeinated beverages (coffee, tea, cola, energy drinks) for at least 4 hours prior to any scheduled PFT or arterial blood gas appointment.
* Dietary Restrictions: Patients undergoing overnight diagnostic sleep studies (polysomnography) conducted at affiliated Meridian sleep suites must avoid heavy meals, alcohol, and sedating medications not explicitly cleared by the attending physician for 6 hours prior to study check-in.

SECTION 4: INSURANCE PLAN COVERAGE MATRIX

4.1 Meridian Health Partners operates four proprietary health maintenance and preferred provider plans. Coverage authorizations for pulmonology services are governed strictly by the following matrix:

Service / Procedure	Meridian Core Plan	Meridian Select Plan	Meridian Premier Plan	Meridian Direct Plan
Outpatient Pulmonology Consultation	Covered (PCP Referral Required)	Covered (Direct Access)	Covered (Direct Access)	Covered (In-Network Only)
Standard PFT (Spirometry & Plethysmography)	Covered	Covered	Covered	Covered
Diagnostic Flexible Bronchoscopy	Covered (Pre-Authorization Required)	Covered	Covered	Covered (Pre-Authorization Required)
Home Sleep Apnea Testing (HSAT Device)	Not Covered	Covered	Covered	Covered
Full In-Lab Polysomnography	Not Covered	Covered (Pre-Authorization Required)	Covered	Not Covered

4.2 Any procedure not explicitly marked as "Covered" within the matrix for a specific plan tier requires an approved exception waiver signed by the Medical Director of Managed Care, Marcus Vance, prior to service delivery.

SECTION 5: REFERRAL TURNAROUND COMMITMENT

5.1 The Pulmonology Department maintains a strict internal operational service level agreement (SLA) for inbound clinical referrals.

5.2 Routine referrals transmitted via the Meridian electronic health record portal must be triaged, reviewed by a mid-level provider or attending physician, and scheduled or formally declined within 48 business hours of receipt.

5.3 Urgent referrals bearing explicit clinical flags for severe dyspnea or suspected malignancy must be triaged and contacted for scheduling within 12 business hours of receipt. Failure to meet these turnaround thresholds triggers an automated administrative review by operations manager Sarah Jenkins.